

Patient agreement to systemic anti-cancer therapy (SACT):

Lenvatinib

Hospital/NHS Trust/NHS Board: _____

Responsible consultant:

Name: _____

Job title: _____

Patient details

Patient's surname/family name: _____

Patient's first name(s): _____

Date of birth: _____

NHS number: _____
(or other identifier)

Special requirements:
(e.g. other language/other communication method)

Name of proposed course of treatment (include brief explanation if medical term not clear)

- ☐ Lenvatinib for the treatment of hepatocellular carcinoma (HCC).
- ☐ Lenvatinib is taken orally once each day. Treatment is supplied every 28 days (one cycle).
- ☐ Treatment will be continued until disease progression or unacceptable toxicity.

Where the treatment will be given

☐ Outpatient ☐ Day unit/case ☐ Inpatient ☐ Other: _____

Statement of health professional

(to be filled in by health professional with appropriate knowledge of proposed procedure, as specified in the hospital/Trust/NHS board's consent policy)

☒ Tick all relevant boxes

☐ I confirm the patient has capacity to give consent.

☐ I have explained the course of treatment and intended benefit to the patient.

The intended benefits (there are no guarantees about outcome)

- ☐ **Curative** – to give you the best possible chance of being cured.
- ☐ **Disease control/palliative** – the aim is not to cure but to control or shrink the disease. The aim is to improve both quality of life and survival.
- ☐ **Adjuvant** – therapy given after surgery/radiotherapy to reduce the risk of the cancer coming back.
- ☐ **Neo-adjuvant** – therapy given before surgery/radiotherapy to shrink the cancer, allow treatment and reduce the risk of the cancer coming back

Statement of health professional

(continued)

Patient identifier/label

Significant, unavoidable or frequently occurring risks

Common side effects:

More than 10 in every 100 (>10%) people have one or more of the side effects listed:

- ☐ Tiredness and feeling weak (fatigue), headache.
- ☐ Diarrhoea, constipation, feeling sick (nausea), being sick (vomiting).
- ☐ Sore mouth and ulcers, appetite loss, dry mouth, indigestion, taste changes, weight loss, tummy (abdominal) pain.
- ☐ Higher blood pressure than normal.
- ☐ Protein in the urine (picked up in urine tests).
- ☐ Soreness, redness and peeling on palms of hands and soles of feet, skin rash, hair thinning or loss.
- ☐ A higher risk of infection from a drop in white blood cells (neutropenia), especially urinary tract infections.
- ☐ Bruising or bleeding due to low platelets (sometimes including haemorrhage).
- ☐ A hoarse voice.
- ☐ Difficulty sleeping, dizziness (which may affect your ability to drive or use machinery).
- ☐ High cholesterol levels, changes in how the liver works (monitored), changes in thyroid hormone levels, low calcium, potassium, magnesium levels.
- ☐ Muscle, joint or back pain, a build-up of fluid in the ankles or legs with weight gain.

Occasional side effects:

Between 1 and 10 in every 100 (1-10%) people have one or more of these effects:

- ☐ Cancer and treatment with Lenvatinib can increase your risk of a blood clot (thrombosis), causing pain, redness and swelling in a leg, breathlessness or chest pain. Tell your doctor straight away if you have any symptoms.
- ☐ A higher risk of a stroke. Seek medical attention straight away if you have symptoms of a drooping face, numbness or weakness on one side of the body, feeling confused or difficulty speaking.
- ☐ Changes in how the heart works (heart attack, heart failure and rhythm changes).
- ☐ Changes in how the kidneys work (which are monitored).
- ☐ Thickening of the skin.
- ☐ Dehydration.

Occasional side effects continued:

- ☐ Passing wind (flatulence).
- ☐ An abnormal opening between the end of the bowel and the skin near to the anus (anal fistula). Inform your doctor or nurse if you experience a change in bowel movements, discharge, or pus from the anus.

Other risks:

- ☐ Changes in the brain (posterior reversible encephalopathy syndrome) causing seizures, confusion, headache, changes in vision.
- ☐ A rare but serious risk of developing jaw problems (osteonecrosis) with Lenvatinib. Healthy bone tissue in the jaw becomes damaged and dies. Gum disease, problems with your dentures, and some dental treatments (such as having a tooth removed) can increase the risk of this. See your dentist before starting Lenvatinib.
- ☐ Lenvatinib may slow wound healing. You may be asked to stop Lenvatinib before an operation or any procedures, including dental work. Please discuss this with your doctor or nurse.
- ☐ Potential side-effects with the anti-sickness may include: diarrhoea, constipation, headaches, indigestion, difficulty sleeping and agitation.
- ☐ Changes in your memory, concentration or ability to think clearly. There can be many causes of this including your treatment, diagnosis, or both.
- ☐ Some anti-cancer medicines can damage ovaries and sperm. This may lead to infertility in and/or early menopause.
- ☐ Some anti-cancer medicines may damage the development of a baby in the womb. It is important not to become pregnant or make someone else pregnant during treatment and for 1 month afterwards. Use effective contraception during this time. Talk to your doctor or nurse.
- ☐ Complications of treatment can very occasionally be life threatening and may result in death. The risks are different for every individual. Potentially life threatening complications include those listed on this form, but, other, exceedingly rare side-effects may also be life threatening.

Statement of health professional

(continued)

Patient identifier/label

Any other risks and information

- ☐ I have discussed the intended benefit and risks of the recommended treatment, and of any available alternative treatments (including no treatment).
- ☐ I have discussed the side effects of the recommended treatment, which could affect the patient straight away or in the future, and that there may be some side effects not listed because they are rare or have not yet been reported. Each patient may experience side effects differently.
- ☐ I have discussed what the treatment is likely to involve (including inpatient / outpatient treatment, timing of the treatment, blood and any additional tests, follow-up appointments etc) and location.
- ☐ I have explained to the patient, that they have the right to stop this treatment at any time and should contact the responsible consultant or team if they wish to do so.
- ☐ I have discussed concerns of particular importance to the patient in regard to treatment (please write details here): _____

Clinical management guideline/Protocol compliant (please tick):

☐ Yes ☐ No ☐ Not available

If No please document reason here: _____

The following written information has been provided:

- ☐ Information leaflet for Lenvatinib.
- ☐ 24 hour alert card or SACT advice service contact details
- ☐ SACT treatment record (cruk.org/treatment-record)
- ☐ Other, please state: _____

Health professional details:

Signed: _____

Date: _____

Name (PRINT): _____

Job title: _____

Statement of interpreter (where appropriate)

Interpreter booking reference (if applicable): _____

I have interpreted the information above to the patient to the best of my ability and in a way in which I believe they can understand.

Signed: _____ Date: _____

Name (PRINT): _____

Job title: _____

Statement of patient

Patient identifier/label

Please read this form carefully. If your treatment has been planned in advance, you should already have your own copy of the form which describes the benefits and risks of the proposed treatment. If not, you will be offered a copy now. If you have any further questions, do ask – we are here to help you. You have the right to change your mind at any time, including after you have signed this form.

☐ I have **had enough time to consider** my options and make a decision about treatment.

☐ I **agree** to the course of treatment described on this form.

A witness should sign below if the patient is unable to sign but has indicated their consent. A person with parental responsibility will be asked to sign for young people under the age of 16 years.

Patient's signature: _____

Name (PRINT): _____ Date: _____

Person with parental responsibility/witness' signature: _____

Name (PRINT): _____ Date: _____

Copy accepted by patient: yes / no (please circle)

Confirmation of consent

(health professional to complete when the patient attends for treatment, if the patient has signed the form in advance)

On behalf of the team treating the patient, I have confirmed that the patient has no further questions and wishes the course of treatment/procedures to go ahead.

Signed: _____

Date: _____

Name (PRINT): _____

Job title: _____

Important notes: (tick if applicable)

☐ See also advance decision to refuse treatment

☐ Patient has withdrawn consent
(ask patient to sign /date here)

Signed: _____

Date: _____

Further information for patients

Contact details (if patient wishes to discuss options later):

Contact your hospital team if you have any questions about cancer and its treatment.

Cancer Research UK can also help answer your questions about cancer and treatment. If you want to talk in confidence, call our information nurses on freephone **0808 800 4040**, Monday to Friday, 9am to 5pm. Alternatively visit **www.cruk.org** for more information.

These forms have been produced by Guy's and St. Thomas' NHS Foundation Trust as part of a national project to support clinicians in ensuring all patients are fully informed when consenting to SACT.

The project is supported by Cancer Research UK. This does not mean you are taking part in a clinical trial.



**CANCER
RESEARCH
UK**

Guidance for health

professionals (to be read in conjunction with the hospital's consent policy)

Patient identifier/label

What a consent form is for

This form documents the patient's agreement to go ahead with the treatment you have proposed. It is not a legal waiver – if patients, for example, do not receive enough information on which to base their decision, then the consent may not be valid, even though the form has been signed. Patients are also entitled to change their mind after signing the form, if they retain capacity to do so. The form should act as an aide-memoir to health professionals and patients, by providing a checklist of the kind of information patients should be offered, and by enabling the patient to have a written record of the main points discussed. In no way should the written information provided for the patient be regarded as a substitute for face-to-face discussions with the patient.

The law on consent

See the following publications for a comprehensive summary of the law on consent. Consent: Patients and doctors making decisions together, GMC 2020 (www.gmc-uk.org/guidance). Reference guide to consent for examination or treatment, Department of Health, 2nd edition 2009 (www.doh.gov.uk).

Who can give consent

Everyone aged 16 or over is presumed to have the capacity to give consent for themselves, unless the opposite is demonstrated. For young people, it is good practice to involve those with parental responsibility in the consent discussions, unless specifically asked not to. A person with parental responsibility must sign this form for a child or young person under the age of 16. Such patients should be given the opportunity to 'assent' to treatment if they wish. If a patient has the capacity to give consent but is physically unable to sign a form, you should complete this form as usual and ask an independent witness to confirm that the patient has given consent orally or non-verbally.

When NOT to use this form

If the patient lacks the capacity to give consent, you should use an alternative form available for this purpose (dependent on patient age). A patient lacks capacity if they have an impairment or disturbance of the brain, affecting the way their mind works. For example, if they cannot do one of the following:

- understand information about the decision to be made
- retain that information in their mind
- use or weigh this information as a part of their decision making process, or

- communicate their decision (by talking, using sign language or any other means)

You should always take all reasonable steps (for example involving more specialist colleagues) to support a patient in making their own decision, before concluding that they are unable to do so.

Relatives cannot be asked to sign a form on behalf of an adult who lacks capacity to consent for themselves, unless they have been given the authority to do so under a Lasting Power of Attorney or as a court deputy.

Information

Information about what the treatment will involve, its benefits and risks (including side-effects and complications) and the alternatives to the particular procedure proposed, is crucial for patients when making up their minds. The courts have stated that patients should be told about 'significant risks which would affect the judgement of a reasonable patient'. 'Significant' has not been legally defined, but the GMC requires doctors to tell patients about 'significant, unavoidable or frequently occurring' risks. In addition if patients make clear they have particular concerns about certain kinds of risk, you should make sure they are informed about these risks, even if they are very small or rare. You should always answer questions honestly. Sometimes, patients may make it clear that they do not want to have any information about the options, but want you to decide on their behalf. In such circumstances, you should do your best to ensure that the patient receives at least very basic information about what is proposed. Where information is refused, you should document this on the consent form or in the patient's notes.

NHS Scotland

NHS Scotland staff should refer to Healthcare Improvement Scotland. Guidance on consent for SACT and local NHS Board guidance on consent aligned to the Scottish legal framework.

References

1. Summary of Product Characteristics (SmPCs) for individual drugs: <https://www.medicines.org.uk/emc>
2. Cancer Research UK: <https://www.cancerresearchuk.org/about-cancer/cancer-in-general/treatment/cancer-drugs>
3. Macmillan Cancer Support: <https://www.macmillan.org.uk/information-and-support/treating/chemotherapy/drugs-and-combination-regimens>
4. Guy's and St. Thomas' NHS Foundation Trust, Chemotherapy consent